

Patient: James Carter, 55-year-old male

Admission date: 2026-05-24

Discharge date: 2026-05-28

Admission reason: Fever, productive cough, fatigue, and shortness of breath

Discharge diagnosis: Community-acquired pneumonia, improving

Hospital course:

James Carter was admitted for community-acquired pneumonia after 4 days of fever, cough with yellow sputum, fatigue, and shortness of breath. Chest X-ray showed right lower-lobe pneumonia. He received IV antibiotics, fluids, and breathing treatments. His fever improved, oxygen level remained stable on room air, and he is stable for discharge home.

Discharge medications:

1. Amoxicillin/clavulanate 875/125 mg by mouth twice daily for 5 days.
2. Azithromycin 250 mg by mouth once daily for 2 more days.
3. Albuterol inhaler: 2 puffs every 4-6 hours as needed for wheezing or shortness of breath.
4. Acetaminophen 500 mg by mouth every 6 hours as needed for fever or discomfort. Do not exceed 3,000 mg per day.

Home instructions:

Rest, drink fluids, and avoid smoking or secondhand smoke. Finish all antibiotics even if feeling better. Use the albuterol inhaler only as directed for wheezing or shortness of breath. Light activity is okay as tolerated, but avoid strenuous activity until breathing and energy improve.

Follow-up:

See primary care clinician in 3-5 days to confirm symptoms are improving and review medications. Repeat chest imaging may be considered in 6-8 weeks or sooner if symptoms do not improve.

Return precautions:

Seek emergency care for severe trouble breathing, chest pain, blue lips, confusion, fainting, or oxygen level below 90% if using a home pulse oximeter. Call the clinician for fever above 38.6 C, worsening cough, inability to keep fluids down, rash, severe diarrhea, or symptoms not improving after 48-72 hours.

Allergies:

No known drug allergies listed.

Discharge condition:

Stable for discharge home.